

Nivolumab versus Everolimus in Advanced Renal-Cell Carcinoma

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BACKGROUND: Nivolumab, a programmed death 1 (PD-1) checkpoint inhibitor, was associated with encouraging overall survival in uncontrolled studies involving patients with previously treated advanced renal-cell carcinoma.

RESULTS: The median overall survival was 25.0 months (95% confidence interval [CI], 21.8 to not estimable) with nivolumab and 19.6 months (95% CI, 17.6 to 23.1) with everolimus. The hazard ratio for death with nivolumab versus everolimus was 0.73 (98.5% CI, 0.57 to 0.93; $P=0.002$).

CONCLUSION: Among patients with previously treated advanced renal-cell carcinoma, overall survival was longer and fewer grade 3 or 4 adverse events occurred with nivolumab than with everolimus.

CLINICAL IMPACT: Opdivo proves significant survival benefits in metastatic cancers, applied directly to protocols for our critical patients.